

WA Health 2027 Annual RMO/SMR Centralised Recruitment

Application Selection Criteria Response

Applicant: Dr Deva Subasic de Azevedo

Job Reference: 493164

Criterion 1 — Eligible for registration with the Medical Board of Australia

I am an Australian citizen holding AHPRA Limited Registration (MED0004001841), with the AMC MCQ and English language requirements completed. Monash Health has submitted an application for Level 2 supervision on my behalf. I understand the conditions that come with limited registration and take seriously the responsibility to practise within approved scope, credentialing requirements and supervision structures.

Criterion 2 — Minimum 6 months clinical/procedural experience in an Australian acute hospital or comparable acute hospital setting within the last 3 years

My Australian clinical exposure was completed through the Monash Health MiPreP program across three rotations: Emergency Department at Clayton, Aged Care and Rehabilitation at Kingston, and Renal Ward at Clayton. These rotations gave me direct experience of Australian hospital systems, Cerner EMR, clinical handover, escalation pathways and the communication standards expected in supervised Australian practice. They also grounded my broader background in Emergency Medicine, Intensive Care, Coronary ICU, Neuro ICU, General Practice and perioperative care within the Australian clinical environment. My procedural experience includes airway management, endotracheal intubation, emergency cardioversion and defibrillation, central venous and arterial line insertion, arterial blood gas sampling, chest tube drainage, ECG interpretation, simple suturing and resuscitation support, all applied within appropriate scope and under supervision as required.

Rotations at Monash Health: Emergency Department, Clayton (15 April 2025 – 15 May 2025) | Aged Care/Rehabilitation, Kingston (25 May 2026 – 5 June 2026) | Renal Ward, Clayton (4 June 2025 – 4 July 2025).

Criterion 3 — Communication, organisation and time management

Communication and organisation have always been central to how I work. At Municipal Hospital Dr Moyssés Deutsch in São Paulo — a referral service covering approximately one million people — I worked as part of a senior team across approximately 20 ED beds, communicating with patients and families under pressure, coordinating with nursing and senior clinical staff, supervising junior doctors and managing referral, admission and discharge decisions across competing priorities. Safe handover, early escalation and clear documentation were non-negotiable in that environment, and they remain so for me now.

Criterion 4 — Professional development and safe practice within personal limitations

I completed Advanced Trauma Life Support and Paediatric Advanced Life Support in 2025, the HEAL AMC Clinical Bridging Course through Victoria University in 2022, and 65 CPD points in 2026. I am aware that experience built in other systems needs to be applied carefully within the Australian context. I am

comfortable working at RMO level under supervision, and I see this as the right foundation for building a sustainable clinical career here.

Criterion 5a — Genuine interest in rural health service delivery

I have a genuine interest in rural and regional placements through WA Country Health Service. I worked previously as the sole doctor during 14-day offshore shifts on a gas extraction platform in Brazil, which required independent thinking, careful judgement and knowing when and how to escalate with limited immediate backup. That experience shaped how I approach resource-limited environments and reinforced the importance of preparation, clear communication and safe escalation when specialist support is not immediately available.

Criterion 5b — Understanding of rural and metropolitan healthcare differences

Rural practice is different from metropolitan practice — broader presentations, less immediate specialist access, longer retrieval pathways and closer reliance on the wider multidisciplinary team, including nursing staff, allied health and retrieval services. I understand this and would approach a WACHS placement with flexibility, respect for supervision structures and a commitment to safe, patient-centred care.